PSEUDOEPHEDRINE

Raises BP
Controlled sale

CHEMICAL NAME (1S,2S)-2-(methylamino)-1-phenylpropan-1-ol (hydrochloride or sulfate)
CLASS Sympathomimetic nasal decongestant (oral)
TRADE NAMES Sudafed · Actifed (with triprolidine) · Clarinase (with loratadine) · Zyrtec-D

THERAPEUTIC USES / INDICATIONS

- Nasal and sinus congestion (common cold, allergic rhinitis, sinusitis).
- Eustachian tube congestion (otitis media with effusion, air travel).

PHARMACODYNAMICS

- Releases **noradrenaline** and stimulates **α-adrenergic receptors** → vasoconstriction of nasal mucosa → ↓ swelling and congestion.
- Mild β effects → ↑ HR.

ADMINISTRATION

- **PO** tablets or syrup; ER tablets swallowed whole.
- Avoid late evening dose (insomnia).
- Sold under restriction in many countries (misuse for methamphetamine).

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Insomnia, restlessness, nervousness.
- Tachycardia, palpitations.
- Dry mouth, headache.

ADVERSE EFFECTS (SERIOUS)

- Hypertension, arrhythmias, stroke, MI (rare).
- Urinary retention.
- Ischaemic colitis; severe skin reactions (AGEP, rare).
- Psychosis/hallucinations (children).

DOSAGES

- **Adult & ≥12 yr:** 60 mg every 4–6 h; max 240 mg/day (ER 120 mg every 12 h).
- **Children 6–12 yr:** 30 mg every 4–6 h (max 120 mg/day).
- Use for max 5–7 days.

CONTRAINDICATIONS & CAUTIONS

- Severe hypertension, coronary artery disease.
- MAOIs within 14 days.
- Severe renal impairment; phaeochromocytoma.
- Children <6 yr; **caution:** hyperthyroidism, diabetes, glaucoma, BPH.

KEY INTERACTIONS

- **MAOIs, linezolid:** hypertensive crisis.
- **Antihypertensives, β-blockers:** ↓ effect / hypertension.
- **Other sympathomimetics:** additive.

PHARMACOKINETICS

- **Onset:** 30 min. **Duration:** 4–6 h (ER 12 h).
- Minimal metabolism.
- Excreted unchanged in urine (↑ with acidic urine). **t½:** 5–8 h.

NURSING MANAGEMENT

- Check BP and cardiac history before advising.
- Monitor urinary symptoms in men with BPH.
- Assess duration of use.

PATIENT EDUCATION

- Do not use if you have high BP or heart disease without medical advice.
- Avoid taking near bedtime.
- Use for no more than a week.
- Check labels; many cold remedies contain it.

KEY POINTS

- Oral α-agonist decongestant.
- Never with MAOIs.
- Raises BP and HR — avoid in hypertension.
- Max 240 mg/day; short-term only.

SOURCES CHECKED **[498]** Benadryl Allergy Relief Plus Decongestant Capsules (UK SmPC) **[477]** 21 CFR Part 341 (US OTC monograph)
Full references p. 432–457